

HANLIM VASOPRESSIN INJECTION 20 IU/ML

FOR I.M/I.V/S.C USE
(VASOPRESSIN 20IU/ML)

【 Appearance】

Clear, colorless solution for Injection: 20 IU/mL; packaged as 1 mL per ampoule
After dilution: Clear, colorless solution

【 Composition】

Active: Vasopressin..... 20 IU/mL

Antimicrobial preservative: Chlorobutanol.....0.05 mg/mL

Other excipients: sodium chloride, glacial acetic acid, water for injection

【 Pharmacodynamics 】

Pharmacotherapeutic group: Vasopressin and analogues

ATC code: H01BA01

Mechanism of action

The antidiuretic action of Vasopressin is ascribed to increase in reabsorption of water by the renal tubules. Vasopressin can cause contraction of smooth muscle of the gastrointestinal tract, gall bladder, urinary bladder and all parts of the vascular bed, especially the capillaries, small arterioles and venules with less effect on the smooth musculature of the large veins. The direct effect on the contractile elements is neither antagonised by adrenergic blocking agents nor prevented by vascular denervation.

【 Pharmacokinetics 】

Absorption

Following subcutaneous or intramuscular administration of Vasopressin injection, the duration of antidiuretic activity is variable, but effects are usually maintained for 2–8 hours.

Biotransformation

The majority of the dose of Vasopressin is metabolised and rapidly destroyed in the liver and kidneys. Vasopressin has a plasma half-life of about 10–20 minutes.

Elimination

Approximately 5% of a subcutaneous dose of Vasopressin is excreted unchanged in the urine four hours after dosing.

【 Indication 】

Vasopressin Injection is indicated:

- to prevent or control polydipsia, polyuria and dehydration in patients with cranial diabetes insipidus caused by a deficiency of endogenous antidiuretic hormone. It is ineffective in nephrogenic diabetic insipidus.
- in treatment of bleeding oesophageal varices.

【 Recommended Dose 】

Diabetes Insipidus

Adult: Vasopressin is given 5 to 20 units, I.M. or S.C every 4 hours.

Bleeding oesophageal varices

Adult: For initial control of variceal bleeding, vasopressin should be given in a dose of 20 units in 100ml of glucose 5% w/v, and infused intravenously over 15 minutes.

*Intravenous infusions are often supplemented with nitroglycerin infusions to minimize cardiac effects.

Excessive dose may cause undesirable effects such as abdominal cramps, nausea and blanching of skin, which though not serious, may be alarming to the patients. Drinking 1 or 2 glasses of water at the time of vasopressin administration may reduce the occurrence of these symptoms. These side effects probably will disappear within few minutes.

Dosing adjustment in hepatic impairment patients: Some patients with cirrhosis responded to lower doses.

【Route of Administration 】

For intramuscular, subcutaneous or intravenous infusion

【 Contraindications 】

- Hypersensitivity to the active substance or to any of the excipients listed
- Anaphylaxis to the active substance or to the excipients listed
- Patient with vascular disease (especially disease of coronary arteries) or those intended to receive halogenated anaesthetic agents.
- Chronic nephritis with nitrogen retention contraindicates the use of vasopressin 20IU/ml injection until reasonable nitrogen blood levels have been attained.

【 Special Warnings and Precautions for Use 】

This drug should not be used in patients with systemic hypertension or vascular disease, especially disease of the coronary arteries, except with extreme caution.

In such patients, even small doses may precipitate pain, and with larger doses, the possibility of the myocardial infarction should be considered. If this drug must be used in patients with peripheral vascular disease then the skin should be observed carefully for signs of ischaemia.

Vasopressin may produce water intoxication. The early signs of drowsiness, listlessness and headaches should be recognised to prevent terminal coma and convulsions.

Adjustment of dosage in cases immediately post-hypophysectomy should be controlled on the basis of measurements of urine osmolality.

Vasopressin should be used cautiously in the presence of epilepsy, migraine, asthma, heart failure or any state in which a rapid addition to extracellular water may produce hazard for an already overburdened system.

Regular monitoring of blood urea nitrogen (BUN) levels is required in patients with chronic nephritis to ensure an adequate level is maintained.

Paediatric population

Not recommended in children below 18 years.

【 Interactions with Other Medicaments 】

The following drugs may potentiate the antidiuretic effect of vasopressin when used concurrently: carbamazepine, chlorpropamide, clofibrate, fludrocortisone, urea or tricyclic antidepressants.

The following drug may decrease the antidiuretic effect of vasopressin when given concurrently: demeclocycline, noradrenaline, lithium, heparin, alcohol.

Ganglion blocking agents may produce a marked increase in sensitivity to the pressor effect of vasopressin.

Severe bradycardia and heart block leading to asystole has been reported in a patient given combined vasopressin and cimetidine therapy.

Incompatibilities

This medicinal product must not be mixed with other medicinal products.

【 Pregnancy and Lactation 】

Oxytocic effect in third trimester has been reported. However, Vasopressin has been used successfully during pregnancy for the treatment of diabetes insipidus.

with no adverse effects on the foetus being reported. Nevertheless, as with all medicines, use during pregnancy should be avoided if possible and the potential benefit to the patient weighed against any possible risk to the foetus.

Breast-feeding

Vasopressin has been administered to breast-feeding women without apparent adverse effect on the infant.

Fertility

No data available.

【 Side Effects 】

The following undesirable effects have been observed and reported during treatment with Vasopressin with the following frequency:

Not known– cannot be estimated from the available data

System Order Class	Adverse effect
Immune system disorders	Hypersensitivity, Anaphylaxis
Metabolism and nutrition disorders	Hyperhydration/ Water intoxication
Nervous system disorders	Headache, Vertigo, Tremor
Cardiac disorder	Chest pain due to angina, Cardiac arrest
Vascular disorders	Peripheral ischaemia, Pallor, Hypertension
Respiratory, thoracic and mediastinal disorders	Bronchospasm
Gastrointestinal disorders	Flatulence, nausea, vomiting, diarrhoea, abdominal pain
Skin and subcutaneous tissue disorders	Gangrene, Hyperhidrosis, Urticaria
Renal and urinary disorders	Fluid retention
General disorders and administration site conditions	Non-cardiac chest pain

【 Symptoms and Treatment of Overdose 】

Management

If water intoxication occurs, no fluids should be given. In severe cases, small amounts of hypertonic saline may be administered. Urea and mannitol infusions may be helpful in cases of cerebral oedema. If a patient should experience angina pain after administration of Vasopressin, amyl nitrite by inhalation or glyceryl trinitrate sublingually, may be given.

【 Effects on Ability to Drive and Use Machine 】

Vasopressin can have an influence on driving as it may cause vertigo

【 Shelf Life 】

36 months.

【 Instructions for Use 】

For Diabetes Insipidus, 5 to 20 units is given by I.M. or S.C every 4 hours.

For bleeding oesophageal varices, 20 units diluted in 100ml glucose 5% w/v and infused intravenously over 15 minutes. Discard unused diluted solution.

【 Storage Condition 】

Store at 2 – 8 °C.

Protect from light. Do not freeze.

Keep container in the outer carton.

Keep out of reach of
children Jauhi daripada
kanak-kanak.

【 Nature and contents of container 】

1 ml USP type 1 colourless, transparent snap off glass ampoule with
green ring. 1 mL ×10 Amp

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